
BOARD BRIEFING / HEALTHCARE TRANSFORMATION

Haven Paediatric Centre

Operational turnaround, culture, and growth

Building the culture, standards, and financial discipline that let a strong young facility grow with confidence. What the numbers show, the plan, and exactly what it costs.

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A strong young facility, ready to build for what comes next

Haven funds its own salaries, holds a steady patient base, and has earned a real reputation in Ikeja. For a facility this young, that is a genuine achievement.

Leadership is now doing what the best operators do: looking hard at the systems beneath the surface before growth outpaces them.

As Haven scales into more complex care, including neonatal intensive care, its clinical governance, its standards of work, and its financial discipline need to be deliberate rather than assumed. This engagement builds exactly that foundation, so the facility grows with confidence.

Culture is the multiplier, not a later add-on.

Process fixes rarely hold on their own. What makes safe, efficient care automatic, rather than dependent on individual diligence, is the culture, the standards of work, and the incentives beneath it. Build that foundation first, and everything above it holds.

Build the foundation first.

Then safety, margin, and growth all follow.



One root cause, showing up in three places

- **Safety.** Consistent, shift-level clinical routines are the backbone of reliable care, and formalising them is foundational as the facility grows.
- **Margin.** Margins are thin despite reasonable revenue, because no ownership culture is pushing efficiency and yield.
- **The answer is already on the table.** Two items already on the board's own decision list, the staff commission structure and the JDS and KPIs, are exactly the incentive and accountability tools a strong culture runs on.

So the work is not to patch processes. It is to build the culture, standards of work, and incentives that make good processes self-sustaining, then reengineer the processes so the culture has something solid to run on.



The cash problem is a working-capital problem, not a profit problem

Leadership feels there is “barely anything left” after salaries. The report shows why: the money is not missing. It is on the shelf and in the HMO ledgers.

~N4.2M

HMO receivables, Leadway and NEM alone, close to a full period's revenue

~N2.77M

Sitting in pharmacy stock, cash converted into inventory on the shelf

~N7M

Total working capital locked up, recoverable without selling a single extra service



NICU is the growth engine, and it rewards getting the basics right first

The highest-yield lever in the building

At an N3M deposit per admission against three beds, nothing else in the facility compares for revenue per bed. This is where growth comes from.

And what safe scaling asks first

Scaling neonatal intensive care durably means putting the governance, standards, and team readiness in place first. Get that right, and the growth compounds.

Get the governance right first. Then scale NICU with confidence, and the revenue follows.



The reporting layer is immature, and that is itself a finding

- Visit-type counts do not reconcile to total encounters.
- The receivables table does not foot.
- A 201% pharmacy “profit” is a markup, not a margin.

None of this is incompetence. It is the absence of a senior operator. You cannot manage what you cannot measure reliably, and right now the numbers the board sees cannot be fully trusted.



One board-led turnaround, five workstreams

1

Diagnostic audit (4 weeks)

Clinical governance, operations, finance and working capital, procurement, HMO economics, reporting. Two quick wins from week one: a checklist-governed crash cart, and a receivables push.

2

Culture, incentives & clinical standards of work, the spine

Shift-level routines, nursing standards of work, the safety-huddle cadence. Redesign the commission structure and JDS/KPIs around quality and ownership, not activity alone.

3

Process reengineering & operations

Vendor-managed inventory (recommend Medbury Pharma) to end stockouts and lift pharmacy margin. Build the receivables-recovery process and a reliable reporting layer.

4

Revenue & growth optimisation

Internal: NICU activation, pricing, pharmacy attach, HMO yield. External: corporate tie-ups, school partnerships (Toddler Town), a referral pipeline.

5

Board-level oversight (optional retainer)

Up to two days a month of partner-level oversight, continuing after the core project. The ongoing governance layer, offered as an opt-in.



A senior operations leader, running the facility day to day

One thing advised early still holds, and leadership is now ready for it: Haven needs a senior, experienced operator running the facility day to day.

- **Define first, then hire.** The audit specifies the role and its KPIs before anyone is recruited, so the hire lands into clarity rather than chaos.
- **Sourced through CadreHealth.** Consult for Africa's workforce platform sources and vets the candidate, including diaspora and senior-operator profiles.



Stabilise fast, build deliberately, then grow

Stabilise Weeks 1 to 2	Crash-cart standard and checklist, receivables recovery push
Diagnostic audit Weeks 1 to 4	Full audit across all functions
Culture & process build Months 2 to 4	Standards of work, incentives, procurement, reporting
Growth Months 3 to 6	NICU activation, pricing, corporate and HMO tie-ups
Oversight Ongoing (optional)	Board-level management retainer



Full value, one visible discount, then the net

Every workstream is priced at standard rates so the board sees the real value, then a single 40% partner discount line, then the net. Nothing is hidden in the rate.

Workstream	Standard	Partner (40% off)
1. Diagnostic audit (4 weeks)	N3,500,000	N2,100,000
2. Culture, incentives & standards	N5,000,000	N3,000,000
3. Process reengineering & operations	N4,000,000	N2,400,000
4. Revenue & growth optimisation	N4,500,000	N2,700,000
Core project total	N17,000,000	N10,200,000

The discount is an N6,800,000 concession, shown in full rather than buried in a lower headline rate.

Optional oversight (WS5): N1,000,000/mo standard, N600,000/mo partner, up to two days a month. Opt-in.



Cost tracks delivery, covered by the cash the early work unlocks

A mobilisation fee on signing, then the balance spread in equal monthly instalments. The receivables recovery and margin gains from the early work comfortably cover the schedule.

Payment	When	Amount
Mobilisation (diagnostic audit)	On signing	N2,100,000
Monthly instalment (x5)	Months 1 to 5	N1,620,000 each
Core project total		N10,200,000

The optional oversight retainer, if taken up, is billed monthly and separately from the schedule above.

A note on governance

Dr Odulana has been invited to Haven's board, and Consult for Africa would be a paid partner. That overlap is common and workable. Pricing and the discount are disclosed in full to all owners, the engagement is clean fixed fees plus an optional retainer, and there is no success fee tied to outcomes he would oversee. Transparency is part of the standard we are setting.



Six months from now

- A safety culture where the crash cart, and every critical routine, is checked every shift by habit, not reminder.
- The N4.2M in receivables recovered, with working-capital discipline that keeps cash moving.
- A vendor-managed inventory model that ends stockouts and lifts pharmacy margin.
- NICU admissions growing safely against a governance standard the team trusts.
- A senior operations leader in post, running a facility that measures itself honestly.
- Incentives that reward quality and ownership, so the culture sustains itself after we step back.

NEXT STEP

Commission the diagnostic audit.

If the board is content with the direction, the first step is the four-week diagnostic audit at N2,100,000 on signing. Its findings shape the detail of the culture, process, and growth work, and the timeline firms up around real data. On the go-ahead, we mobilise within the week.

Consult for Africa

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